

## Common Medical Abbreviations

| ABBREVIATION    | MEANING                               |
|-----------------|---------------------------------------|
| a.c.            | Before meals                          |
| ADL             | Activities of daily living            |
| Ad lib          | As desired                            |
| AIDS            | Acquired immunodeficiency syndrome    |
| ASHD            | Atherosclerotic heart disease         |
| a.u.            | In the ear                            |
| b.i.d.          | Twice a day                           |
| BP              | Blood pressure                        |
| bpm             | Beats per minute                      |
| BUN             | Blood urea nitrogen                   |
| C               | Celsius, Centigrade, complement       |
| c *             | With                                  |
| Ca              | Calcium                               |
| CA              | Cancer                                |
| CAD             | Coronary artery disease               |
| caps            | Capsules                              |
| CBC             | Complete blood count                  |
| CC, C.C.        | Chief complaint                       |
| cc              | Cubic centimeter                      |
| CHF             | Congestive heart failure              |
| Cl              | Chloride, Chlorine                    |
| cm              | Centimeter                            |
| CNA, NAC        | Certified Nursing Assistant           |
| CNS             | Central nervous system                |
| CO              | Carbon monoxide, Cardiac output       |
| CO <sub>2</sub> | Carbon dioxide                        |
| COMP            | Compound                              |
| COPD            | Chronic obstructive pulmonary disease |
| CPR             | Cardiopulmonary resuscitation         |
| C&S             | Culture and sensitivity               |
| cu              | Cubic                                 |
| CVA             | Cerebrovascular accident              |
| D&C             | Dilatation and curettage              |
| DIFF, diff      | Differential blood count              |
| DM              | Diabetes mellitus                     |
| DX, Dx, dx      | Diagnosis                             |
| ECG             | Electrocardiogram                     |

|                  |                                          |
|------------------|------------------------------------------|
| ED               | Emergency department                     |
| EEG              | Electroencephalogram                     |
| EMG              | Electromyogram                           |
| ENT              | Ear, nose and throat                     |
| exc              | Excision                                 |
| F                | Fahrenheit                               |
| FBS              | Fasting blood sugar                      |
| ft               | Foot                                     |
| FUO              | Fever of unknown origin                  |
| G, gm            | Gram                                     |
| GI               | Gastrointestinal                         |
| gr               | Grain                                    |
| gt, gtt          | Drop, drops                              |
| GU               | Genitourinary                            |
| GYN, gyn         | Gynecology                               |
| Hb               | Hemoglobin                               |
| HCl              | Hydrochloric acid, hydrochloride         |
| HCO <sub>3</sub> | Bicarbonate                              |
| HCT              | Hematocrit                               |
| Hg               | Mercury                                  |
| Hgb              | Hemoglobin                               |
| HR               | Heart rate                               |
| Hr               | Hour                                     |
| h.s.             | At bedtime, hour of sleep                |
| Hx               | History                                  |
| ICU              | Intensive care unit                      |
| I.M.             | Intramuscular                            |
| inj              | Inject                                   |
| I&O              | Intake and output                        |
| IPPB             | Intermittent positive pressure breathing |
| IU               | International unit                       |
| IUD              | Intrauterine device                      |
| I.V.             | Intravenous                              |
| K                | Potassium                                |
| kcal             | Kilocalorie (food calorie)               |
| kg               | Kilogram                                 |
| l                | Liter                                    |
| lb               | Pound                                    |
| lt               | Left                                     |
| LPN, LVN         | Licensed Practical (Vocational) Nurse    |
| LUQ              | Left upper quadrant                      |
| m                | Meter, minim                             |
| MD               | Medical Doctor                           |
| mEq              | Millequivalent                           |

|          |                                                                       |
|----------|-----------------------------------------------------------------------|
| Mg       | Milligram                                                             |
| mcg      | Microgram                                                             |
| MI       | Myocardial infarction                                                 |
| ml       | Milliliter                                                            |
| m        | Micron                                                                |
| mcl      | Microliter                                                            |
| mm       | Millimeter                                                            |
| mo       | Month                                                                 |
| MRI      | Magnetic resonance imaging                                            |
| MS       | Multiple sclerosis, morphine sulfate                                  |
| N        | Nitrogen, normal (strength of solution)                               |
| Na       | Sodium                                                                |
| NaCl     | Sodium chloride                                                       |
| NGT      | Nasogastric tube                                                      |
| NPO      | Nothing by mouth                                                      |
| o.d.     | Right eye                                                             |
| o.s.     | Left eye                                                              |
| o.u.     | Both eyes                                                             |
| OTC      | Over the counter (a drug that can be obtained without a prescription) |
| oz       | Ounce                                                                 |
| P        | Phosphorus, pressure, pulse                                           |
| p.c.     | After meals                                                           |
| Perrla   | Pupils equal, round, react to light and accommodation                 |
| P.O.     | By mouth                                                              |
| PPD      | Purified protein derivative (of tuberculin)                           |
| ppm      | Parts per million                                                     |
| p.r.n.   | As needed, whenever necessary                                         |
| Pro time | Prothrombin time                                                      |
| psi      | Pounds persquare inch                                                 |
| PT       | Physical therapy                                                      |
| q        | Every                                                                 |
| q.d.     | Every day                                                             |
| q.h.     | Every hour                                                            |
| q2h      | Every 2 hours                                                         |
| q4h      | Every 4 hours                                                         |
| q.i.d.   | Four times a day                                                      |
| R        | Respirations                                                          |
| RLQ      | Right lower quadrant                                                  |
| RN       | Registered nurse                                                      |
| R/O      | Rule out                                                              |
| ROM      | Range of motion                                                       |
| RUQ      | Right upper quadrant                                                  |

|                |                                  |
|----------------|----------------------------------|
| Rx             | Prescription                     |
| s*             | Without                          |
| S.C., SQ, subq | Subcutaneous                     |
| sec            | Second                           |
| SOB            | Short of breath                  |
| spec           | Specimen                         |
| sp gr          | Specific gravity                 |
| S&S            | Signs and symptoms               |
| ss*            | One-half                         |
| stat           | Immediately                      |
| sx             | Symptoms                         |
| T              | Temperature                      |
| Tab            | Tablets                          |
| TB             | Tuberculosis                     |
| Temp.          | Temperature                      |
| TIA            | Transient ischemic attack        |
| t.i.d.         | Three times a day                |
| TPN            | Total parenteral nutrition       |
| TPR            | Temperature, pulse, respirations |
| tsp            | Teaspoon                         |
| UA             | Urinalysis                       |
| ung            | Ointment                         |
| URI            | Upper respiratory infection      |
| Vfib           | Ventricular fibrillation         |
| VS             | Vital signs                      |
| UTI            | Urinary tract infection          |
| WBC            | White blood cell                 |

**9.2 Staffing Guidelines ..... Revised 05/01/2018 Reviewed 05/01/2018**

**Policy Purpose**

A written staff schedule is developed and implemented with consideration given to the specific needs of the residents and the responsibilities of each position. State requirements will be incorporated into staffing schedules.

**Definitions**

**Caregiver:** A team member who assists residents with personal care, which may include medications. They may also assist with housekeeping, laundry, meal service and activity needs (Universal worker).

**Guidelines**

**Staffing**

The Resident Care Director is responsible for overseeing the staffing needs in the care departments. The resident Care Director reports to the General Manager.

A staffing tool is used to determine the appropriate labor hours based on census and resident care needs. The tool incorporates resident service levels. When state requirements specify staffing levels or ratios, a tool incorporating those requirements should be used if different from Merrill Gardens' standard staffing model. The tools should be used to complete the monthly care staffing schedule and reviewed regularly to adjust the schedule for changes in census and acuity.

At least one appropriately trained caregiver is to be on duty at all times.

**AZ: (803.C.1.h)** *The Community will schedule an appropriate level of staffing to meet our residents' needs.*

1. *Department Managers will post a schedule for their department prior to the beginning of each schedule period.*
2. *Staffing levels will be determined based upon the current staffing tool and take into account resident needs.*
3. *Care staff schedules will be retained for 1 year from the end of the schedule period.*

**Responsible Manager:** Department Managers

**FL: Minimum staffing**

| Residents      | 0-5 | 6-15 | 16-25 | 26-35 | 36-45 | 46-55 | 56-65 | 66-75 | 76-85 | 86-95 |
|----------------|-----|------|-------|-------|-------|-------|-------|-------|-------|-------|
| Hours per week | 168 | 212  | 253   | 294   | 335   | 375   | 416   | 457   | 498   | 539   |

*For every 20 residents over 95, add 42 staff hours per week.*

**GA:** There will be a minimum on-site staff to resident ratio of one awake direct care staff person per 15 residents during waking hours.

There will be a minimum of one awake direct care staff person per 25 residents during non-waking hours where the residents have minimal care needs.

Flexible care ratios will be utilized when there is one on one care by a sitter or outside provider, but minimum state ratios of 1:15 and 1:25 will always be in place.

Staff, such as cooks and maintenance staff, whose duties do not include routine oversight or delivery of direct personal care are not counted in the ratio.

**NV:** At least one employee must be awake and on duty at all times, and an additional employee must be able to provide care within ten minutes after being informed that their services are needed.

**SC:** In each building, there shall be at least one staff member/direct care volunteer for each eight residents or fraction thereof on duty during all periods of peak hours. In each building, during non-peak hours, there shall be at least one staff member/volunteer on duty for each thirty residents or fraction thereof. They must be awake and dressed at all times and able to appropriately respond to resident needs during non-peak hours. There shall be a staff member available on each floor at all times residents are present on that floor.

#### **Care Team Schedule**

A written schedule for care team members will be created and available. The schedule should be made available at least 2 weeks in advance. Generally, the schedule is created monthly and adjusted as needed. Changes in the schedule are to be approved by the Resident Care Director, General Manager or Manager on Duty. The staffing scheduled should be retained for a year after the period covered, then may be destroyed.

#### **Replacement of Open Shifts**

When the schedule is created, team members will be offered open shifts. Assignment will be at the discretion of the Resident Care Director or General Manager. Consideration will be given to controlling overtime when multiple team members request a shift.

Requests for time off are to be made in accordance with the Team Member Handbook. A request is not a guarantee that those specific dates will be granted. We will attempt to accommodate requests for time off, but not to the detriment of the ability to schedule sufficient staff to meet the needs of the residents.

Open shifts that occur due to call-offs should be reported to the Resident Care Director, General Manager or Manager on Duty for direction. Based on resident needs, if a call-off needs to be replaced, a team member may be asked to contact off-duty caregivers to fill the shift. Other options to consider are reassignment of scheduled team members to cover the vacancy, splitting shifts, or contacting nearby communities. In some cases, agency staff may be utilized upon approval of the Vice President of

Operations. The General Manager has authority to do what is necessary to help ensure sufficient staff are available to meet the needs of the residents.

### **Garden House Staffing**

The Garden House Supervisor is responsible for overseeing the staff working in the Garden House. The Garden House Supervisor reports to the General Manager. The General Manager may determine whether the Resident Care Director or the Garden House Supervisor creates the staffing schedule for the Garden House.

All team members working in the Garden House are universal workers responsible for personal care/ADLs, laundry, meal service, housekeeping and activities.

A staffing tool specifically designed for the Garden House is used to determine the appropriate level of staffing. Generally, Garden House staffing is managed through staffing ratios. The ratios are incorporated into the Garden House staffing tool. When state requirements specify staffing levels or ratios, a tool incorporating those requirements should be used if different from Merrill Gardens' standard staffing model.

The physical layout of the GH and its proximity to the rest of the community should be considered to enable staff to monitor residents and provide services.

**AL:** A Registered Nurse is required to complete GH admission assessments, change in condition assessments, monthly assessments, and formulate written interventions to address identified care problems. At least two staff members must be on duty at all times. The following GH staffing ratio must be met:

| <b>Staff Number</b> | <b>7 AM - 3 PM</b>                                                            | <b>3 PM - 11</b>                                                                | <b>11 PM - 7 AM</b>                                                             |
|---------------------|-------------------------------------------------------------------------------|---------------------------------------------------------------------------------|---------------------------------------------------------------------------------|
| 2                   | 1 - 16 Residents                                                              | 1 - 16 Residents                                                                | 1 - 16 Residents                                                                |
| 3                   | 17 - 24 Residents                                                             | 17 - 36 Residents                                                               | 17 - 48 Residents                                                               |
| 4                   | 25 - 32 Residents                                                             | 37 - 48 Residents                                                               | 49 - 64 Residents                                                               |
| 5                   | 33 - 40 Residents                                                             | 49 - 60 Residents                                                               | 65 - 80 Residents                                                               |
| 6                   | 41 - 48 Residents                                                             | 61 - 72 Residents                                                               | 81 - 96 Residents                                                               |
| 7                   | 49 - 56 Residents                                                             | 73 - 84 Residents                                                               | 97 - 112 Residents                                                              |
| 8                   | 57 - 64 Residents                                                             | 85 - 96 Residents                                                               | 113 - 128 Residents                                                             |
| 9                   | 65 - 72 Residents                                                             | 97 - 108 Residents                                                              | 129 - 144 Residents                                                             |
| 10                  | 73 - 80 Residents                                                             | 109 - 120 Residents                                                             | 145 - 160 Residents                                                             |
| 11                  | 81 - 88 Residents                                                             | 120 - 132 Residents                                                             | 161 - 176 Residents                                                             |
| 1 Additional Staff  | For each 8 residents, or any fraction thereof, by which the census exceeds 88 | For each 12 residents, or any fraction thereof, by which the census exceeds 132 | For each 16 residents, or any fraction thereof, by which the census exceeds 176 |

**GA:** Staffing Ratios for the Garden House will be 1:6-9 during waking hours and 1:12-15 during sleeping hours dependent on resident needs especially in regard to self-preservation.

**NV:** No more than six residents to each caregiver during hours when residents are awake.





# ASSISTED LIVING STAFFING MODEL

The AL staffing model accounts for the time required to provide care services to residents by considering acuity, number of residents, and size of community. Complete the areas highlighted in blue every two weeks.

| Care Levels    | Resident Count | Min. Points In Care Level | Max. Points In Care Level | Minutes Per Resident Day | Hours Per Day | Hours Per Week |
|----------------|----------------|---------------------------|---------------------------|--------------------------|---------------|----------------|
| Care Level 1   |                | 1                         | 125                       | 0                        | 0.0           | 0.0            |
| Care Level 2   |                | 126                       | 225                       | 0                        | 0.0           | 0.0            |
| Care Level 3   |                | 226                       | 325                       | 0                        | 0.0           | 0.0            |
| Care Level 4   |                | 326                       | 425                       | 0                        | 0.0           | 0.0            |
| Care Level 5   |                | 426                       | 525                       | 0                        | 0.0           | 0.0            |
| Care Level 6   |                | 526                       | 625                       | 0                        | 0.0           | 0.0            |
| Care Level 7   |                | 626                       | 725                       | 0                        | 0.0           | 0.0            |
| Care Level 8   |                | 726                       | 825                       | 0                        | 0.0           | 0.0            |
| Care Level 9   |                | 826                       | 925                       | 0                        | 0.0           | 0.0            |
| Care Level 10  |                | 926                       | 1025                      | 0                        | 0.0           | 0.0            |
| Care Level 11  |                | 1026                      | 1125                      | 0                        | 0.0           | 0.0            |
| Care Level 12  |                | 1126                      | 1225                      | 0                        | 0.0           | 0.0            |
| Care Level 13  |                | 1226                      | 1325                      | 0                        | 0.0           | 0.0            |
| Care Level 14  |                | 1326                      | 1425                      | 0                        | 0.0           | 0.0            |
| Care Level 15  |                | 1426                      | 1525                      | 0                        | 0.0           | 0.0            |
| Care Level 16  |                | 1526                      | 1625                      | 0                        | 0.0           | 0.0            |
| Care Level 17  |                | 1626                      | 1725                      | 0                        | 0.0           | 0.0            |
| Care Level 18  |                | 1726                      | 1825                      | 0                        | 0.0           | 0.0            |
| Care Level 19  |                | 1826                      | 1925                      | 0                        | 0.0           | 0.0            |
| Care Level 20  |                | 1926                      | 2025                      | 0                        | 0.0           | 0.0            |
| Resident Count | 0              |                           |                           | 0                        | 0.0           | 0.0            |

## Additional Office Hours

|   |     |     |     |
|---|-----|-----|-----|
| 0 | 0.0 | 0.0 | 0.0 |
|---|-----|-----|-----|

## Staffing Adjustments based upon Community Size:

How Many Floors Does Your Community Have? (If there is more than 1 building add the total number of floors).  
 How Many Units Does Your Community Have? (Excluding Garden House and Cottages).  
 How many resident do you have that are not receiving care services?

|   |     |     |     |
|---|-----|-----|-----|
| 0 | 0.0 | 0.0 | 0.0 |
| 0 | 0.0 | 0.0 | 0.0 |
| 0 | 0.0 | 0.0 | 0.0 |

## HOW SHOULD YOU STAFF YOUR AL DEPARTMENT?

| Total Caregiver Hours | Minutes Per Resident Day | Hours Per Day | Hours Per Week |
|-----------------------|--------------------------|---------------|----------------|
| 7am To 3pm Staffing   | 849                      | 14            | 99             |
| 3pm to 11pm Staffing  | 849                      | 14            | 99             |
| 11pm to 7am Staffing  | 480                      | 8             | 56             |

\*Whole department staffing hours\* includes hours for caregiving and office hours. Does not include training hours or RSD hours if community has RSD.

\*Minimum staffing requires 1 night care giver and 1 night universal worker. The universal worker is coded to G&A and the hours for that universal worker are not included in this staffing model.

## NOTES

List the count of residents receiving AL care services in the blue boxes by care level. The model calculates hours by assuming that all residents on a level of care are at the Max Points in a care level. (We know that is not actually the case but it gives you more staffing time). We then calculate that each point requires 4.75 minutes per day of staff time.

RCD HOURS PER WEEK NOT INCLUDED BELOW  
40

Not all office hours are captured in the Care Levels listed above. Therefore, this section considers additional office hours based upon count of residents receiving services. For this staffing model, additional office hours are maxed at 8 hours per day.

The AL staffing model considers the size of the community by looking at number of floors, total number of units, and residents who are not receiving care services. This section is intended to consider travel time between units and floors. It also is intended to account for the time spent helping and reevaluating residents who do not receive AL care services.

Every two weeks, use these hours to staff the AL department. For communities with an HSD, it does not consider those hours. RCD time has been removed and is noted up above. Training hours are not considered in this staffing model. The AL staffing model considers two levels of minimum staffing. Less than 30 residents gives a minimum staffing of 42 hours per day. More than 30 residents gives a minimum staffing of 48 hours per day. If the resident acuity causes the number of minutes to be higher than 2880, the AL staffing model will give the total minutes and will ignore the minimum staffing levels. Minimum staffing requires 1 night care giver and 1 night universal worker. The universal worker is coded to G&A and the hours for that universal worker are not included in this staffing model.



## GARDEN HOUSE STAFFING MODEL

As acuity and number of residents receiving care services increases, the staffing model accounts for the time required to provide care services to residents. Complete the areas highlighted in blue every two weeks.

### Staffing Adjustments based upon Garden House size:

How Many Units Does Your Garden House Have?

Is your Garden House Attached/Within the Main Building?

How Many different buildings in your Garden House?

| Care Levels    | Resident Count | Min. Points in Care Level | Max. Points in Care Level |
|----------------|----------------|---------------------------|---------------------------|
| Care Level 1   | 0              | 0                         | 425                       |
| Care Level 2   |                | 426                       | 525                       |
| Care Level 3   |                | 526                       | 625                       |
| Care Level 4   |                | 626                       | 725                       |
| Care Level 5   |                | 726                       | 825                       |
| Care Level 6   |                | 826                       | 925                       |
| Care Level 7   |                | 926                       | 1025                      |
| Care Level 8   |                | 1026                      | 1125                      |
| Care Level 9   |                | 1126                      | 1225                      |
| Care Level 10  |                | 1226                      | 1325                      |
| Care Level 11  |                | 1326                      | 1425                      |
| Care Level 12  |                | 1426                      | 1525                      |
| Care Level 13  |                | 1526                      | 1625                      |
| Care Level 14  |                | 1626                      | 1725                      |
| Care Level 15  |                | 1726                      | 1825                      |
| Care Level 16  |                | 1826                      | 1925                      |
| Care Level 17  |                | 1926                      | 2025                      |
| Care Level 18  |                | 2026                      | 2125                      |
| Care Level 19  |                | 2126                      | 2225                      |
| Care Level 20  |                | 2226                      | 2325                      |
| Care Level 21  |                | 2326                      | 2425                      |
| Care Level 22  |                | 2426                      | 2525                      |
| Care Level 23  |                | 2526                      | 2625                      |
| Care Level 24  |                | 2626                      | 2725                      |
| Care Level 25  |                | 2726                      | 2825                      |
| Resident Count | 0              |                           |                           |

The GH staffing model accounts for how many buildings make up your Garden House.

List the count of residents receiving GH care services in the blue boxes by care level. This model adds additional time for every resident that is level 12+.

| HOW SHOULD YOU STAFF YOUR GH DEPARTMENT?                    | Daily Hours | Weekly Hours |
|-------------------------------------------------------------|-------------|--------------|
| Caregiver Hours Only (w/out GHD, GHS, or PPD)               | 48          | 336          |
| Suggested Caregiver Staffing Hours without GHD, GHS, or PPD |             |              |
| Caregiver Hours 7am to 3pm Approx.                          | 16          | 112          |
| Caregiver Hours 3pm to 11pm Approx.                         | 16          | 112          |
| Caregiver Hours 11pm to 7am Approx.                         | 16          | 112          |

The below ratios DO NOT include a GHD, GHS or the PPD.

Morning shift caregiver minimums should be 1 to 6 staffing ratio.

Afternoon shift caregiver minimums should be 1 to 10 staffing ratio. Minimum staffing of 2 caregivers on duty if 20 residents or less. Minimum staffing of 3 caregivers on duty if 21 residents or more.

For NOC shift, the caregiver minimum should be 1 to 15 staffing ratio. If GH is detached from the main building there should be a minimum staffing of 2 caregivers on the NOC shift.

\* Housekeeping for Garden House units should be performed by the housekeeping staff. The Housekeeping staffing model includes time for garden house units.



### 9.3 Garden House Staff Development

..... Revised 05/01/2018 Reviewed 05/01/2018

#### Policy Purpose

Individuals with dementia present unique challenges to care providers. In order to understand and meet the needs of these residents, staff members require specialized education. All team members who routinely work in the Garden House will participate in the staff development program. The Garden House staff development program is a supplement to the community orientation and training requirements. In the event there are state specific requirements, those will be met.

#### Definitions

**Dementia** – Loss of memory, thinking and reasoning that interferes with everyday life. May be reversible or non-reversible.

#### Guidelines

##### **Documentation of Training**

The General Manager or designee will schedule training for team members as needed. Individual records of completed training modules are maintained in the Learning Management System. A report can be run, or the system logged into to view the completed training.

*GA: The community must maintain documentation reflecting course content, instructor qualifications, agenda and attendance roster for all training provided. Documentation of the continuing education requirements should include:*

- *Topic/title*
- *Date*
- *Instruction time*
- *Instructor's name and qualifications*
- *Summary of the content and attendance*

*Copies of training documentation may be kept in the team members' personnel files or in a separate training file.*

##### **Initial Training**

All regularly scheduled Garden House team members will complete training in the following areas within the first three months of employment unless there are state specific requirements:

- Introduction to dementia; including diagnosis, progression and treatment
- Communicating with dementia residents
- Behaviors; including repetition, wandering, aggression, depression, paranoia and hallucinations

- Personal care for the dementia resident

**AL:** Garden House team members will complete training in the following areas prior to resident contact:

1. State law and rules on assisted living facilities and specialty care assisted living facilities.
2. Identifying and reporting abuse, neglect and exploitation
3. Basic first aid
4. Advance Directives
5. Protecting resident confidentiality
6. Safety and nutritional needs of the elderly
7. Resident fire and environmental safety
8. Understanding the aging mind
9. Basic brain function
10. Common Neuropsychiatric disorders in the elderly
11. Basic evaluation of the dementia patient
12. Cognitive symptoms of dementia
13. Psychiatric symptoms of dementia
14. Behavioral problems associated with dementia
15. End of life issues in dementia
16. Dementia other than Alzheimer's
17. Research and dementia
18. Nutrition and hydration needs of the resident with dementia to include feeding techniques
19. Safety needs of residents with dementia

**AZ: (803.C.1.b)** Team members and volunteers will attend initial orientation.

1. Orientation will be conducted by Merrill Gardens team members and will include participation from Department Managers as their availability allows.
2. New hires will be required to complete orientation and documentation will be completed and kept in the new hire/volunteers file.
3. Orientation will include at a minimum, topics covering:
  - a. Merrill Gardens Policy and Procedures
  - b. Orientation to the needs of the residents
  - c. Merrill Gardens philosophy and goals
  - d. Service Plans-including scope of services
  - e. Infection Control
  - f. Safe Food Handling
  - g. Recognizing and Reporting abuse
  - h. Incident Reporting
  - i. Emergency Preparedness
4. Orientation will consist of videos, verbal instruction, walking tours, demonstrations and quizzes.
5. Resources will include: subject matter experts, online courses and Merrill Gardens material.
6. The Community will conduct monthly training as assigned.
7. Attendees will sign in on a Training Attendance Record.

**Responsible Manager:** Business Office Manager, General Manager, Department Managers

**GA: SCALF** The administrator must strive to ensure that any person working in the community as staff receives the following training prior to resident contact within the first 60 days of employment:

- *Residents' rights and identification of conduct constituting abuse, neglect or exploitation of a resident and reporting requirements to include receipt of a copy of the Long-Term Care Facility Resident Abuse Reporting Act.*
- *General infection control principles including importance of hand hygiene in all settings and attendance policies when ill.*
- *Training necessary to carry out job duties*
- *Emergency preparedness procedures*

**Staff providing hands-on personal care must receive the following initial training within the first 60 days in addition to the training listed above:**

- *Current certification in emergency first aid except where the staff person is a currently licensed health care professional*
- *Current certification in cardiopulmonary resuscitation where the course requires return demonstration*
- *Medical and social needs and characteristics of the resident population, including special needs of resident with dementia*
- *Resident rights, and the provision of care to residents that is individualized and helpful*
- *Training specific to job duties such as permissible assistance with medications, contraindications with medications that must be brought to the attention of appropriate individuals, assisting residents in transferring, ambulation, proper food preparation, proper performance of health maintenance activities if serving as a proxy caregiver, and responding appropriately to dementia-related behaviors.*

**Initial training prior to administering medications and/or providing care to Garden House residents shall include, at minimum:**

- *Certified Medication Aide training for anyone administering medications according to state guidelines. They may provide care to resident but not medication administration until they have been listed on the state site as approved*
- *General training as outlined above*
- *Assisted living philosophy of care for dementia*
- *Policies and procedures related to care and staff responsibilities including wandering, egress control, Maddie's Act*
- *Introduction to common behavioral characteristics and understanding of recommended behavior management techniques.*

**All staff will complete a 5-hour fire safety class within 90 days of hire. This will be taught with a curriculum and instructor approved by the State Fire Marshall's office.**

### **Training Requirements**

**All regularly scheduled Garden House staff will complete training in the following areas within 6 months of employment unless there are state specific requirements:**

- **Activities for the dementia resident-Personal Pathways**
- **Environment**
- **Staff/ Family relationships**
- **Intimacy and sexuality**
- **End of life**

**AL:** All Garden House team members will complete the DETA (Dementia Education and Training Act) Brain Series Training developed by the Alabama Department of Mental Health and Mental Retardation or equivalent approved by the State Health Officer prior to providing any resident care.

**GA: SCALF Training Within 6 Months of Employment**

Within 6 months of employment additional training will occur in the following:

- Common behavioral problems and recommended behavior management techniques
- Nature of Alzheimer's disease and other dementia, including the definition of dementia and knowledge of dementia-specific care needs
- Communication skills that facilitate better resident staff relations
- Positive therapeutic interventions and activities such as exercise, sensory stimulation, activities of daily living skills
- Role of the family in caring for residents with dementia as well as the support needed by the family of these residents
- Environmental modification that can avoid problematic behavior and create a more therapeutic environment
- Development of comprehensive and individual service plan and how to update or provide relevant information for updating and implementing them consistently across all shift, including establishing baseline care needs
- New developments in dementia care that impact the approach to caring for the residents in the special unit
- Skills for recognizing physical or cognitive changes in the resident that warrant seeking medical attention
- Skills for maintaining the safety of resident with dementia

**Continuing Education**

Continuing education opportunities will be made available to Garden House team members. It is expected that team members will take advantage of the training offered. Training will cover topics of interest in the field of dementia care. State specific requirements are to be met.

**AL:** Training will cover topics of interest in the field of dementia care and to remain knowledgeable of the needs of the residents. A Registered Nurse will identify team member refresher training needs and provide or arrange for the needed training. Each team member working in the Garden House will have a minimum of 6 hours of continuing education per year.

**GA: SCALF** During the first year of employment, the administrator and all staff offering hands on personal services to residents must complete at least **twenty-four (24) hours** of continuing education relevant to job duties and include topics such as:

- Cardiopulmonary resuscitation and first aid certification
- Utilizing Standard precautions
- Working with Alzheimer's or other cognitive impairments
- Working with persons with development disabilities or mental illness
- Providing social and recreational activities
- Understanding legal issues
- Performing necessary physical maintenance
- Fire safety



- *Housekeeping activities*
- *Recognizing and reporting abuse, neglect and exploitation*
- *Preparing and serving food safely*
- *Preserving the dignity and rights of residents to make meaningful choices*
- *Providing and documenting medication assistance*
- *Other topics as determined necessary to support compliance*

*Staff assigned to provide personal services in the Garden House must have **eight (8) hours of the 24** related specifically to dementia care.*

***Beginning with the second year of employment, staff providing hands-on personal assistance must have a minimum of sixteen (16) hours of job-related continuing education as referenced above.***

- *Staff assigned to provide personal services in the Garden House must have **two (2) hours of the 16** related specifically to caring for residents with dementia. (refer to GH02 GA for memory care training)*
- *All staff must complete a **2-hour** fire safety class refresher every **three years** from the initial training. The refresher training must have written approval of the state Fire Marshall's office.*

